When Caring Backfires: Experimental Evidence from Family Caregiving in India

Jenny Newman-Wang
MIT

Camille Falézan
MIT

Shrddha Rajesh
J-PAL SA

Girija Vaidyanathan
J-PAL SA

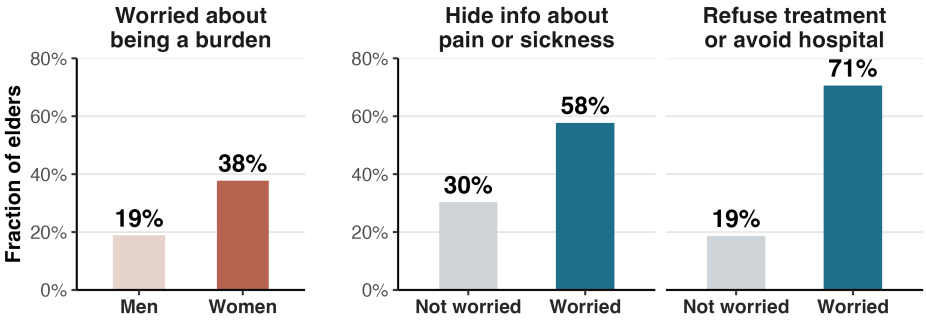
NEUDC 2026

Many chronic diseases go untreated in low-income settings

- Rising chronic disease burden as populations age
  - Projected 2 billion people aged 60+ by 2050, with **80% in LMICs** WHO 2025
- Many recent policies focused on improving supply-side for care
  - Subsidized testing/medication, screening at the doorstep Subramaniam2025
- Despite these efforts, **take-up** for preventative care is still low (Banerjee et al., 2023)
  - In India, 40% (45%) of individuals with diabetes (hypertension) are unaware
  - **Today:** Focus on demand-side barriers

Elders often depend on family to get healthcare

- Barriers may be at the **household-level** for older adults
  - Family caregivers often provide daily support
  - But elders worry about burden and may not ask for help

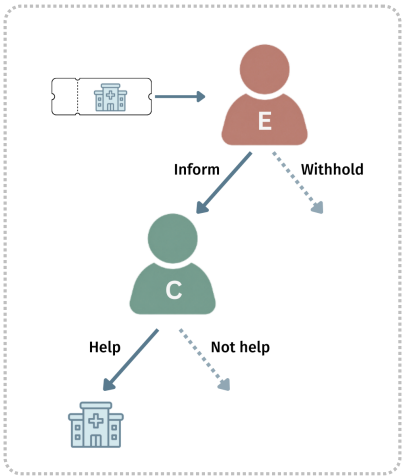


Do intrahousehold information frictions deter take-up of healthcare for elders?

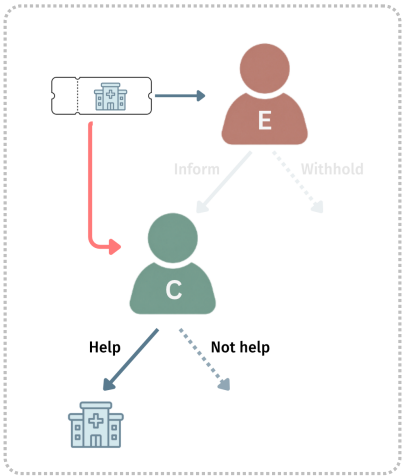
- **Our paper:** Worry about burden $\rightarrow$ Information frictions $\rightarrow$ Take-up of care
  - This can decrease healthcare for elder, even when families value it
  - Context: Health check-ups for elderly women in rural Tamil Nadu, India

Our paper in two pictures

Only elder knows (Status quo)



Both know (Treatment)



Main findings

1. Impact: Information frictions reduce check-up attendance
   - Offer free health check-ups and vary who is informed
   - $\Rightarrow$ Informing caregivers raises attendance, especially for worried elders
2. Mechanism: Worry decreases communication by elders
   - Directly measure elder willingness to share check-up info
   - $\Rightarrow$ Worried elders inform less – and are sensitive to caregiver costs
3. Efficiency: Caregiver help is not motivated by pressure *(Not today)*
   - Understand caregiver motives to help
   - $\Rightarrow$ Decision to help does not change in private vs public

Worried elders forgo care that both elders and caregivers value

Setting: Elderly women and their caregivers in Tamil Nadu, India (N=731 pairs)

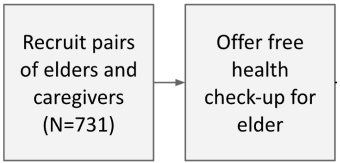
- Elders in our study are women aged 60-75 in rural Tamil Nadu, India
  - **Worried**: 20% report ever forgoing treatment out of worry about burden
- Caregivers are family members who live nearby and support in health tasks
  - Primarily sons (43%) and husbands (21%)

Caregiver Roles | Sample Recruitment | Sample descriptives



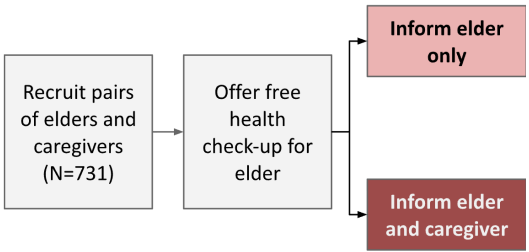


We partner with a private hospital to offer elders a free cardiac check-up



- We offered free cardiac check-ups
  (doctor consult, BP, ECG, QRISK, HbA1c, echo)
  - $150-250 PPP (∼ 3,000-5,000 INR)
  - Not broadly provided by government
- Invites conducted separately, privately
  - Paper voucher with details for elders
  - Sign-up: Caregiver must call by deadline

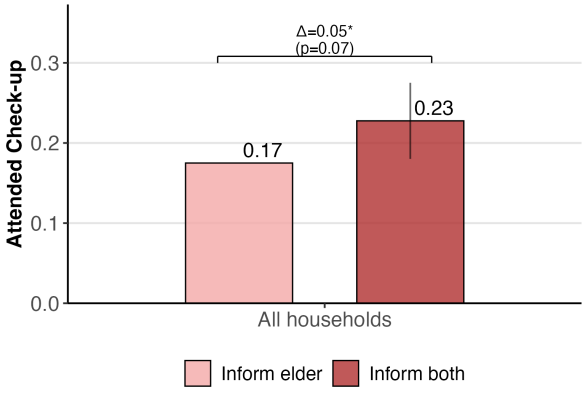
Cross-randomize how costly the check-up is (for caregivers)



Measure take-up of health check-up

(sign-up, attend)

Informing caregiver about check-up increases take-up by 5pp

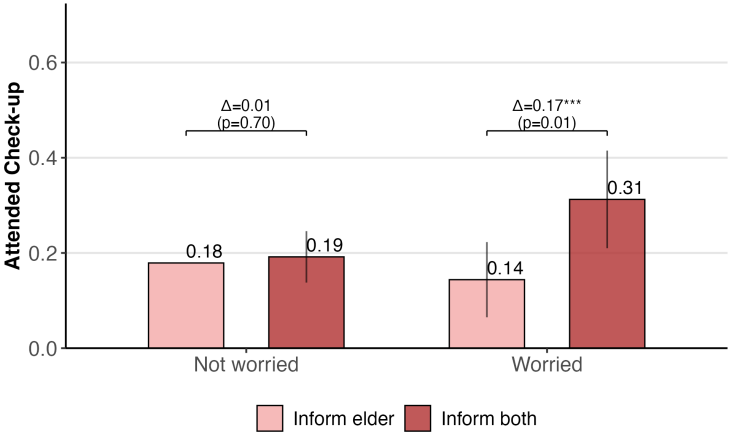


Effects are **large** in magnitude:

26% increase in sign-up rates and 30% in attendance

Effect is 70-73% of effect for free transport intervention

Effect is driven by households with worried elders, whose attendance doubles

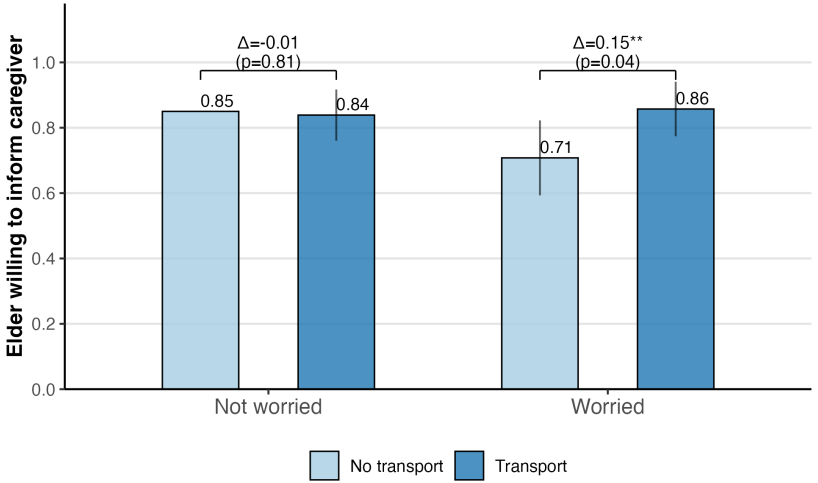


Ruling out alternative explanations

Our results are not explained by...

- Differences in elder or caregiver characteristics Elder traits by worry
- Higher actual costs of attending With interacted controls
- Higher asking costs (rejection sensitivity, logistical) Direct vs Indirect Info Sharing
- Less frequent interaction with caregivers Closeness by worry

Worried elders are 14pp less willing to inform caregivers about check-ups



Upshot: Household experiment shows that caregivers help when informed

- When caregivers are directly informed about a check-up, they act on it
  - But worried elders do not inform them (pressure)
  - This decreases take-up. Is this inefficient? Depends on the caregiver's motives
- We run a second experiment with **caregivers** (N=444) to rule out two concerns:
  - ~~Caregivers may help due to **pressure**~~ Test for pressure
  - ~~Caregiver help now may **crowd out** future help~~ Test for crowdout
- $\Rightarrow$ Elders overestimate pressure imposed by informing the caregiver

Taking stock: Worry about burden captures an important relationship dynamic

- Households are aging, making **caregiving burden a key issue**
  - Elders worry about burden, which decreases health take-up
  - ... despite caregivers being willing to help
- Mutual “caring" can **amplify information frictions and lower health take-up**
  - Concern for others can backfire when others’ preferences unknown
- **Policy can help**: Sharing info directly with caregivers increases take-up
  - Treatment effects are large, comparable to providing free transport

Thank you!

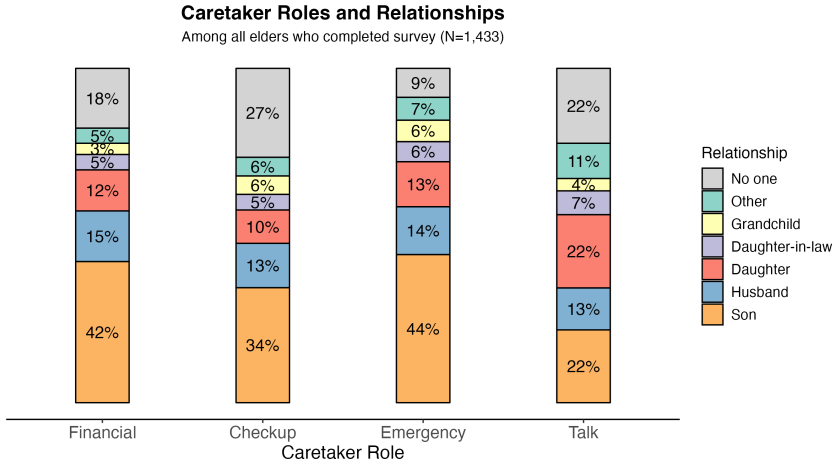
wangjm@mit.edu

Our sample consists of elderly women aged 60-75 in rural Tamil Nadu

- Median age 68 years old, about half are widowed (52%)
- **Most are mobility impaired** (77% difficulty walking >1km, 58% leaving home)
- 82% of elders live with their caregiver
- Most have had some kind of check-up in the last year (83%)
- 16% have gotten heart check-up in the last year
- 93% were **interested in attending** the check-up

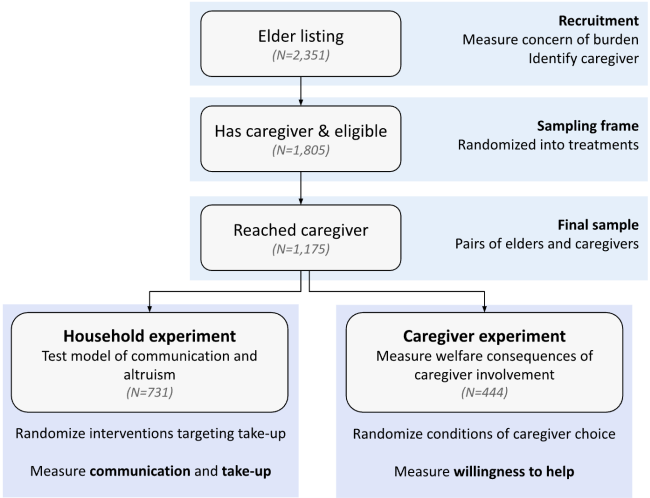
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Elders primarily depend on sons and husbands for health-related support



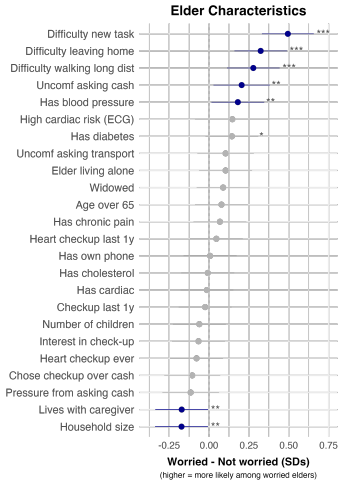
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Sampling: We recruit pairs of elders + caregivers for a household survey



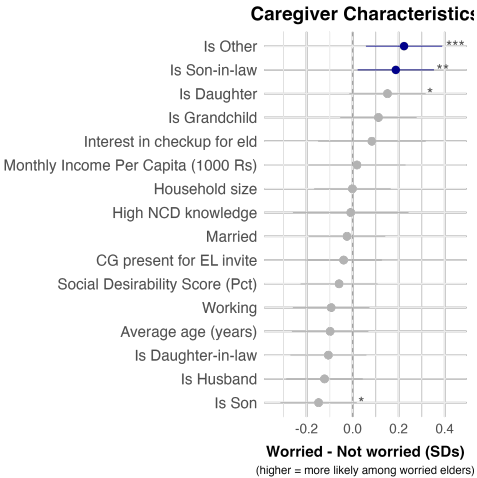
Elders worried about burden are more mobility constrained and dependent

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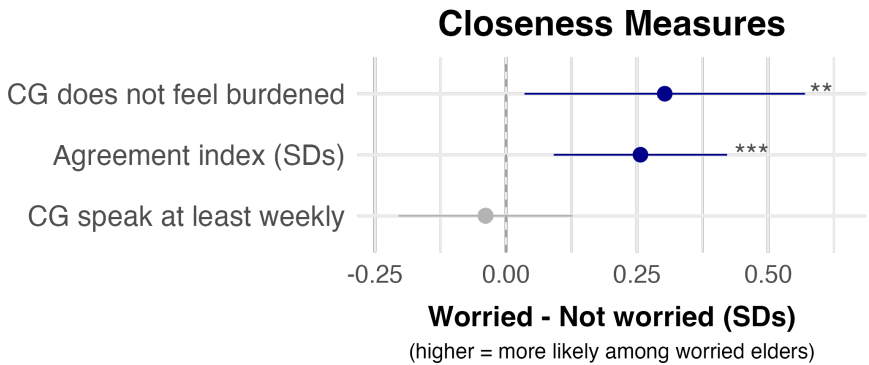
Elders worried about burden have caregivers who are less likely to be sons

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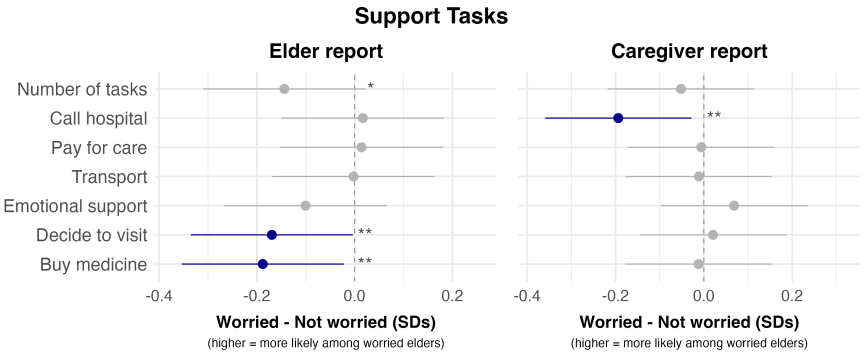
Elders worried about burden have caregivers who are less likely to feel burdened

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Worried elders do not receive more support from caregivers

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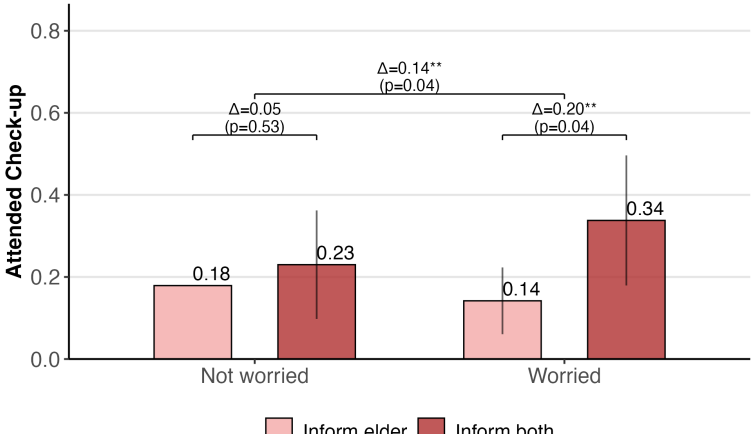
Worried elders do not differ in time to mention health issues (pain, sickness)

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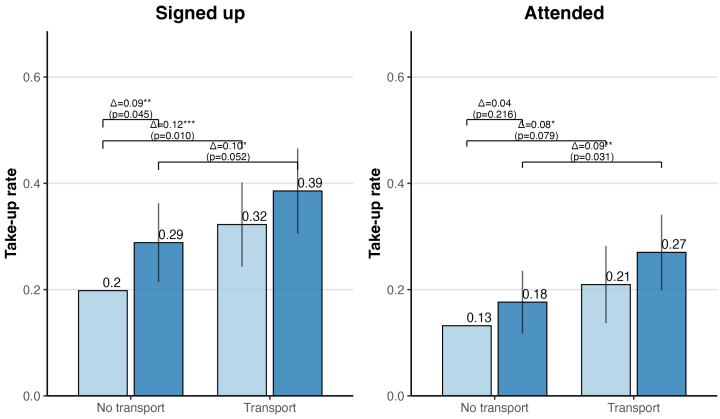
Heterogeneity persists even when controlling for imbalanced characteristics, interacted with treatment

Back



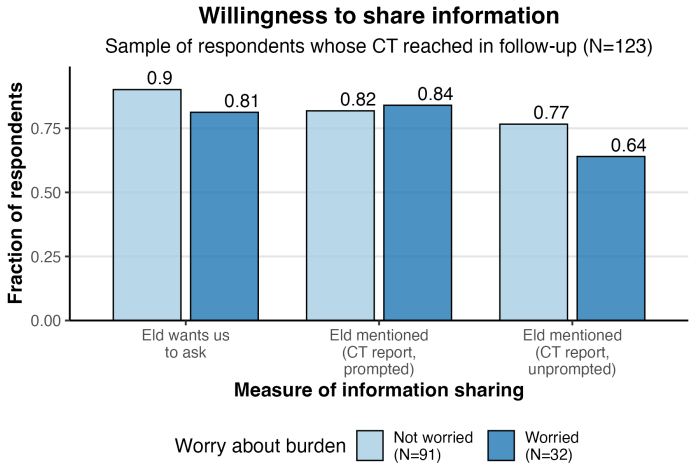
Interaction effect of transport and tell both is positive but not statistically significant

Not powered to test for interaction effects



Direct and indirect willingness to inform rates are similar

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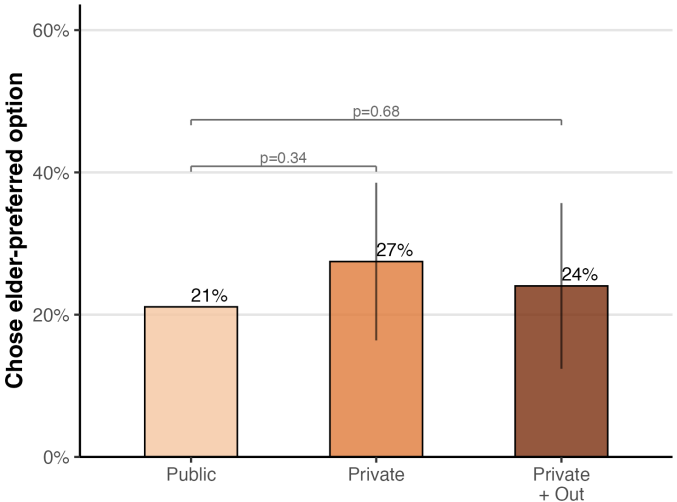


Testing for pressure: We randomize observability of caregiver decision to help

- Offer caregivers a check-up for the elder
- Outcome: Among caregivers who agree, offer two types of **transport support**
  - Choose option preferred by elders (reimbursement)
  - Choose option preferred by caregivers (arranged)
- Randomize condition under which caregiver chooses
  - **Public.** Baseline condition
  - **Private.** Removes social pressure (surveyor, elder)
  - **Private + Out.** Removes social and psychological pressure (e.g. guilt, self-image)[1]

Results Back

Caregivers help at similar rates, even when offered an out

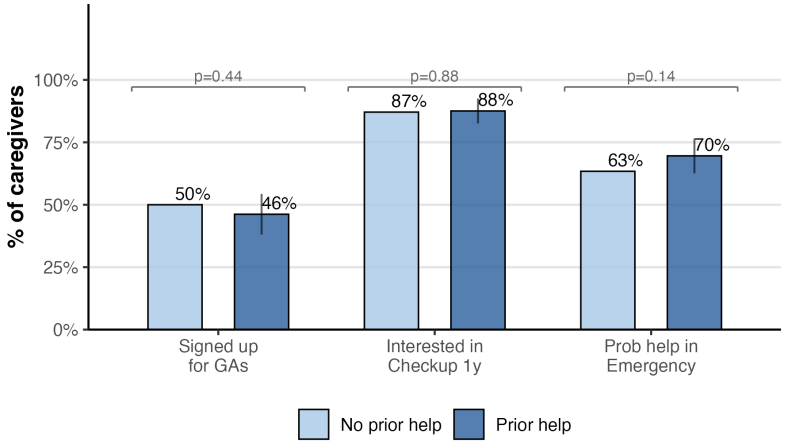


Testing for crowd-out: Randomize whether caregiver has already provided help

- Outcomes: Willingness to help on other tasks
  1. Sign up for group activities (20 Rs)
  2. Want to learn more about general checkup in 1y
  3. Willingness to help in emergency (5-point scale)
- Randomize whether check-up is offered **before** vs **after** measuring outcomes
  - 63% of caregivers sign up for the check-up[2]

Crowd-out results Back

Caregivers do not reduce help in other domains



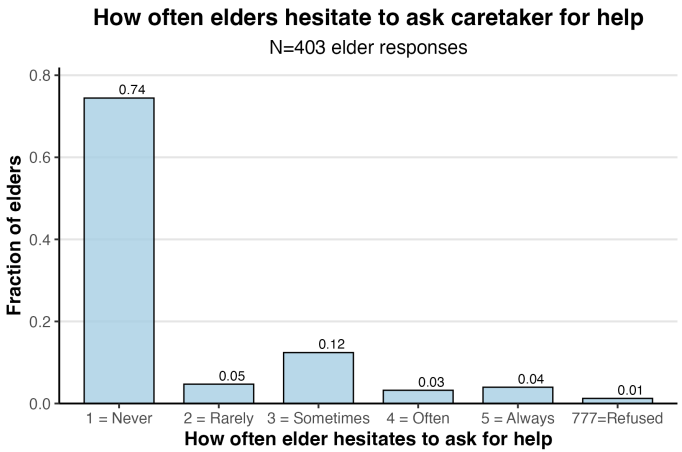
Treatment = invited to checkup before GAs. 90% confidence intervals.

We find no evidence of differential crowd-out for caregivers of worried elders

	Dependent variable:		
	GA Signup	Interested in Checkup 1y	Emerg: Prob help
	(1)	(2)	(3)
Prior Help Committed	−0.037	0.016	0.073
	(0.061)	(0.037)	(0.052)
	p = 0.538	p = 0.674	p = 0.155
Worry	0.045	0.076	0.001
	(0.078)	(0.048)	(0.067)
	p = 0.567	p = 0.113	p = 0.986
Prior Help * Worry	0.012	−0.035	−0.024
	(0.105)	(0.064)	(0.089)
	p = 0.910	p = 0.584	p = 0.792
Control Mean	0.5	0.87	0.63
Strata FEs	Y	Y	Y
Controls (LASSO)	Y	Y	Y
Observations	444	444	444
R^2	0.275	0.326	0.414
Note:			*p<0.1; **p<0.05; ***p<0.01

Back

Most elders report not hesitating to ask when they need help with something (74%)



Among those who ever hesitate to ask, burden is most common reason (70%)

Few mention rationing their asks to the caregiver (8%)

Table: Elder reasons for not asking for help

variable	N	Fraction
I do not want to burden or trouble them	68	0.69
They may be upset with me for asking	36	0.37
I think they will say no	22	0.22
I don’t like to feel dependent	13	0.13
I may need help for something urgent later	8	0.08
Don’t know	1	0.01
Other, specify	1	0.01

Back